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THE PSYCHIATRY OF PHILIPPE PINEL *

EVELYN A. WOODS AND ERIC T. CARLSON **

The great Revolution of 1789, which ushered in a whole new era in French medicine, made possible the emergence of Philippe Pinel as a prominent figure in the medical world of Paris and his consequent work in the field of psychiatry. Prior to 1789 Pinel was almost entirely excluded from the ranks of leading Paris physicians. The " lowness and intrigue " among the medical men of Paris led a disgusted Pinel to avoid them,[1] and the fact that he had not received his medical education at the Paris medical school, moreover, made him an outsider.[2] But his whole scientific attitude was the true obstacle to Pinel's allying himself with medicine's *ancien régime* in Paris, where progress was condemned as heresy, and medicine was taught almost exclusively from the works of Hippocrates and Galen.

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[2] Pinel's first fifteen years in Paris might be termed his apprenticeship. During this time he turned his hand to many enterprises: tutoring, translating, editing, writing, and studying. He did so partly because these intellectual activities were more to his taste than the ceaseless activity of a busy medical practice, and partly because he was unable to obtain the necessary license to practice medicine in Paris. To procure a license an outsider had to appear in competition before the Faculty of Medicine and defend a thesis. If successful, he was granted the degree of doctor-regent and allowed to practice; if not, he was excluded from the ranks of licensed Paris physicians. Pinel appeared in this competition four times, and each time was defeated, not by his lack of knowledge, but by his awkward presentation, for he was not a gifted speaker. A rather ironic anecdote found in several biographies of Pinel recounts the failure of his last attempt to obtain the degree. In this case he was competing against a former gendarme who was defending a thesis on equitation which Pinel had polished to perfection for him in his Montpellier days (René Semelaigne, *Les grands aliénistes français* [Paris: Steinheil, 1894], pp. 28-29).

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progressive in attitude than their Parisian colleagues, they still tended to approach medicine in a philosophical rather than a scientific spirit. The years of Pinel's medical training were years in which various theories, approaches, and techniques struggled for supremacy in the medical world. Although most of the prominent figures in medicine praised the technique of induction advanced by Bacon, in practice they tended to make more use of Cartesian deduction—taking a basic premise, they immediately derived the most general principles from it and went on to propound a multitude of medical theories, interpretations of phenomena, and principles of treatment based on only the most fragmentary evidence. Outstanding among these philosophical systems were such schools of thought as the iatro-mechanism of Borelli, Baglivi, and de Sauvages, the iatrochemistry of Sylvius and Willis, the animism of Stahl, the vitalism of Bordeu and Barthez, and the nervous pathology of Hoffman, Cullen, and Brown.

The attitude of all these systems was opposed in France by the newly-founded ideologist school,[3] which held as its main tenet the proposition that in science theory should be based on accumulation and analysis of abundant data. Originating in Bacon's precepts, their method was derived primarily from Locke's theory of the mind of the newborn as a *tabula rasa* and especially from Condillac's exposition of all mental faculties as originally derived from sensation and modified by association. The ideologists insisted that, since all knowledge came from perception of external phenomena, the only true basis for theory was prolonged and profound observation and analysis of such phenomena—a thorough knowledge of the facts must precede the hypothesis.

Pinel, who had studied science and philosophy at a religious college, received the degree of *maître-ès-arts* in mathematics and earned doctoral degrees in medicine from both the Universities of Toulouse and Montpellier,[4] was fully acquainted with all the opposing theories of the system-builders. He chose, however, to ally himself with the ideologists and whole-heartedly espoused their technique of observation and analysis.

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He made this choice because his extensive medical education had impressed him with the folly of attempting to erect complex, all-inclusive systems on the basis of unsubstantiated hypotheses. A cautious approach prevailed in his own research and kept him from committing himself to the defense of any comprehensive theory of disease. He sought to restrict his research to dealing with " only those symptoms and distinctive signs which are recognizable by the senses and which are not susceptible to vague reasoning." [5] Endowed with great intelligence, ability, and persistence, he was able to advance psychiatric knowledge by his thorough study of these " symptoms and distinctive signs "; his research culminated in his innovations in the treatment of insanity. In other words, he not only freed the study of mental illness from the obscuring theories of the system-builders, but also prepared the way for modern psychiatry.

If Pinel's intellectual abilities made him a capable researcher, his natural benevolence made him a sincere humanitarian. Although personally rather aloof and reserved, he nonetheless felt a profound pity for the wretched inmates of eighteenth-century asylums and was eventually led to make their illness his life study.[6] He himself was convinced that in-

home he was sent to complete his study of the humanities at the Oratorian College in Lavaur. Pinel's intellectual ability won him much attention at Lavaur, and the bishop recognized his potential and persuaded him to take minor orders. After four years, however, Pinel left Lavaur and the priesthood for the University of Toulouse and the study of mathematics and medicine (René Semelaigne, *Aliénistes et philanthropes: Les Pinel et les Tuke* [Paris: Steinheil, 1912], pp. 9-12). Pinel received his M. D. from Toulouse in 1773, but, instead of setting about the active practice of medicine, he chose to enroll at the medical school of Montpellier. This decision was apparently prompted by a general disappointment in the calibre of the faculty and the quantity of the instruction at Toulouse, and his consequent sense of not being fully qualified to practice. Famous for its excellent professors, Montpellier in the eighteenth century was one of the outstanding European medical schools and certainly more progressive than the old Paris Faculté de Médecine. It combined the Greek and Arabic traditions in its approach to medical education and emphasized therapeutics rather than theory (Jules Guiart, *Histoire de la médecine française: Son passé, son présent, son avenir* [Paris: Editions Nagel, 1947], pp. 91-96, 168).

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sanity was a disease as much as any other, and therefore curable in a majority of cases. Many of his contemporaries and predecessors, affected by the widespread humanitarian movement of the period, had held a similar belief about mental illness, but they differed from Pinel in trying to apply the same therapeutic measures used in cases of purely somatic disease to mental illness. Pinel himself strongly believed that in most cases the psychological approach was more efficacious than the physiological; he had worked with patients and found this to be so.

When Pinel was introduced to their society in the mid-1780's, the ideologists—such men as Cabanis, Cousin, Thouret, Destutt de Tracy, Roussel—had relatively little official power in the medical world of Paris, and so Pinel's connection with them had small practical advantage. With the Revolution, however, the republican-inclined ideologists came to dominate much of the new government, and because they held influential government posts they were able to appoint their colleagues to important offices. Aware of Pinel's thorough knowledge of medicine, they appointed him to the faculty of the new Paris school of medicine; knowing his interest in mental illness and his work with the insane, they made him resident physician to two Paris asylums, first Bicêtre and later Salpêtrière.[7] It is for his psychiatric achievement that Pinel is remembered today, and it is with this aspect of his work that this paper will be concerned.

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[7] In doing this, Pinel's friends had given him a Herculean task to perform, for conditions in Paris asylums at the time were deplorable (Jacques Tenon, *Mémoires sur les hôpitaux de Paris* [Paris: Méquignon, 1816], p. 88). Virtual houses of detention for all individuals unable or unwilling to be independent, law-abiding citizens (Paul Courbon, "Pinel psychiatre," *Annales méd.-psychol.*, 1927, 12 s., *2*: 10-11), Bicêtre and Salpêtrière housed the criminal and the indigent as well as the insane. Functioning thus, they were quite naturally custodial institutions. Persons declared insane were first committed to the Hôtel-Dieu in Paris and there submitted to the current forms of vigorous therapy—bleeding, purging, cold showers, etc. (Raymond de Saussure, "French psychiatry of the eighteenth century," *Ciba Symposia*, 1954, *11*: 1224). If after six months or so their condition was not significantly improved—and frequently it was not—they were then committed to the custodial hospitals as incurable (E. Pariset, "Eloge de Philippe Pinel," in his *Histoire des membres de l'Académie Royale de Médicine,* [Paris, Baillière, 1850], vol. 1, p. 227). The men were sent to Bicêtre, which housed about 280 patients, the women to Salpêtrière, where over 600 persons were confined (De Saussure, *op. cit.*, p. 1223). Pinel went to Bicêtre in 1793; two years later he was transferred to Salpêtrière, where he remained until his death in 1826 (Semelaigne, *Alién. et phil.*, p. 55).

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Numerous medical historians have described the European insane asylums in the years before Pinel. In general, conditions were atrocious. There was no provision for cleanliness or comfort, much less for anything resembling therapy (probably because at the time the insane were considered both insensible and incurable), and asylums were custodial institutions rather than therapeutic hospitals. There were exceptions to this situation, but barbarity and ignorance were the rule.

This situation was destined to change, however, for throughout Europe a general movement towards psychiatric reform was developing. Outstanding in this movement were Daquin [8] in southeastern France, Chiarugi [9] in Italy, and Tuke [10] in England, as well as Pinel in Paris. All these men were appalled by their predecessors' failure to recognize the value of kindness and sympathy in the treatment of the insane. In the eyes of the reformers, the outstanding fault of the old practice was the use of chains, and so, virtually simultaneously, they all experimented with unshackling the insane, with according them humane treatment generally, and with using psychological therapy. This new approach, which soon spread through Europe and America, was known as moral treatment.

It was Pinel, however, who became most famous for these psychiatric innovations, and who took his place in the forefront of this simultaneous movement towards enlightened treatment of the insane.[11] Here, as in all his research, Pinel avoided ambitious hypotheses, feeling it possible to be an able psychiatrist—that is, to treat patients successfully—without espousing any precise psychological system. Because of his distaste for theory and hypothesis, Pinel refused to discuss the more abstract questions

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[11] Pinel's prominence is explained by the fact that he worked in Paris, wrote in French, and made a name for himself with his *Nosographie philosophique, ou la méthode de l'analyse appliquée à la médecine* (Paris; 1798). In addition, he avoided making damaging mistakes by refusing to theorize, and he trained a generation of successors who carried on his fame.

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of psychiatry, such as the operation of the mind and the real nature of mental illness.[12]

He concentrated instead on the external characteristics of insanity, and in doing so he encountered a problem which still troubles psychiatry: what method of studying the infinite variations of thought, emotion, and behavior will reveal the basic elements beneath the overt and subjective manifestations of illness? Pinel's solution was, of course, analysis. In attempting to apply analysis to psychiatric research he demonstrated a depth of awareness and understanding of this new scientific method not shared by most of his medical contemporaries and successors, although at the same time he was led into certain incorrect conclusions because of the difficulty of applying analysis to psychiatric material. Pinel was always willing, however, to emend his statements when further experience proved them mistaken.[13]

It was Pinel's belief that by observing numerous patients and by keeping detailed records he would eventually discover the component and fundamental factors of insanity. Consequently he studied and recorded such signs of illness as disordered speech, unusual actions, expression of bizarre emotions, and various somatic changes.[14] In time his researches led him to certain conclusions about the various types of mental illness and its various causes.[15]

After following many cases from onset of illness through recovery, Pinel concluded that there were four general types of insanity:[16] melan-

[12] For purposes of accurate description of the various manifestations of mental illness, Pinel adopted the psychological vocabulary of Locke and Condillac and referred to "the intellectual faculties" and "lesions" thereof, but he declined to base his psychiatry on their psychology which defined insanity as unnatural liaison of logically incompatible ideas. His observation of cases of insanity in which there were no delusions or lesions of judgment (*i. e.* manie sans délire) had proven to him the inaccuracy of this definition. Nor did Pinel join the majority of his medical colleagues in accepting the thesis of Locke and Condillac that the cause of insanity was always some organic lesion of the brain. His autopsies had shown him that insanity could occur without brain damage, and, moreover, that brain damage could occur without insanity.

[13] For example, at first Pinel asserted that in cases of periodical mania only the most powerful medications could have any helpful effect, and that therefore the physician should not hesitate to prescribe them. Further observation, however, demonstrated to him that even periodical mania was better treated with psychological methods than with medications (*Traité*, 2nd ed., p. 368).

[14] *Traité*, 2nd. ed., p. 136.

[15] Pinel's conclusions about cause and diagnosis may be found in both the *Traité* and the *Nosographie philosophique*.

[16] In the last edition of his *Nosographie philosophique* (Paris: J. A. Brosson, 1818) Pinel added four more categories of insanity: hypochrondria, somnambulism, nightmare, and hydrophobia.

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of psychiatry, such as the operation of the mind and the real nature of mental illness.[12]

He concentrated instead on the external characteristics of insanity, and in doing so he encountered a problem which still troubles psychiatry: what method of studying the infinite variations of thought, emotion, and behavior will reveal the basic elements beneath the overt and subjective manifestations of illness? Pinel's solution was, of course, analysis. In attempting to apply analysis to psychiatric research he demonstrated a depth of awareness and understanding of this new scientific method not shared by most of his medical contemporaries and successors, although at the same time he was led into certain incorrect conclusions because of the difficulty of applying analysis to psychiatric material. Pinel was always willing, however, to emend his statements when further experience proved them mistaken.[13]

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cholia, mania, dementia, and idiotism.[17] He recognized, however, that these diagnostic categories were not mutually exclusive, for he had seen patients shift from one to another in the course of their illness.[18] Melancholia was classically characterized as depression, but to Pinel it signified, essentially, a disorder of the intellectual faculties, especially those of perception and imagination, so that the individual's thoughts became fixated on a single idea, usually delusional in character. The melancholic's mood might be extremely elated or depressed; his actions were rarely violent. Judging by Pinel's excellent and detailed clinical descriptions, we today would classify these cases as schizophrenic, affective, paranoid, or organic conditions, and possibly as severe psychoneuroses.

Mania, classically termed fury, was a much more complex illness, whose outstanding characteristic was "strong nervous excitement."[19] This "excitement" usually took the form of irrational violence, and the intellectual faculties and the emotions (or passions) were disturbed. Pinel subdivided mania into "manie avec délire," which might be either periodical or continual, and "manie sans délire," in which the patient was extremely violent but still rational, that is, he still knew who he was and what he was doing.[20]

Certain patients with conditions which we today know as catatonic schizophrenia, chronic regressed schizophrenia, organic brain disease, and mental deficiency, were termed demented or idiotic by Pinel, who considered them generally incurable. In the demented the faculty of association was so impaired that there was no connection between successive ideas, nor between ideas and sense perceptions; although always moving, the patient was incapable of directed activity. The idiot, on the other hand, was sunk in a continual state of apathy caused by partial or total obliteration of the emotions and intellectual faculties.[21]

[17] At Bicêtre, 40% of the patients suffered from "manie avec délire," 7.5% from "manie sans délire," 30% from idiotism, 13.5% from melancholia, and 9% from dementia.

[18] Philippe Pinel, *A Treatise on Insanity*, D. D. Davis, tr. (Sheffield: W. Todd, 1806), pp. 172-173. This work will be referred to hereafter as Davis Translation.

[19] *Ibid.*, p. 159.

[20] Because patients suffering from "manie sans délire" retained their powers of understanding even during their spells of violence, Pinel originally placed them in a fifth category, but further research demonstrated to him that in all other respects these patients manifested the characteristics of true mania, and he re-organized his nosological system accordingly (*Traité*, 2nd. ed., p. 138, footnote). In his article "Pinel's conception of the psychopathic state," *Bull. Hist. Med.*, 1949, *23*: 461-468, Jerome Kavka identifies "manie sans délire" with what today's terminology describes as the psychopathic or sociopathic personality.

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Pinel initially expressed scorn for the scholastic concern with causes of mental illness and presented his own ideas on the subject in an incidental, scattered fashion. Later, however, he himself chose to make aetiology a major topic for discussion. He divided causes into two basic species: predisposing and occasional. Comprised in the former were heredity, an extremely irregular mode of life, and what he termed the spasmodic, oppressive, and expansive passions. Among the numerous occasional causes were alcoholism, hypochrondria produced by excesses, brusque suppression of internal hemorrhage or skin eruptions, menopause, aftereffects of various fevers, gout, and a violent blow on the head. The logic of his analytic approach had brought him to the brain as the seat of illness, but he was sufficiently clear-headed to recognize that, when his autopsies revealed no organic pathology, he would have to rely on his records and observations of a case. This approach in turn led him to emphasize the patient's way of life and his emotions as causative factors in his illness.[22]

Far more significant than his general concepts about classification and cause, however, are Pinel's contributions to psychiatric treatment. He began by attempting to learn the true character of a disease and "to find what helpful resources nature could produce" [23] by allowing the disease to proceed unmolested by indiscriminate medical treatment and unobscured by pre-formed bias. Having achieved a thorough knowledge of the various manifestations of mental illness, Pinel felt he was ready to

results of Pinel's dependency upon symptoms was his failure to recognize the difference between mental deficiency and the degenerated psychotic state. Both states had similar external manifestations, and so he considered them similar in nature. It remained for his successor, Esquirol, to clarify this point.

[22] Some of Pinel's "occasional causes" (*e.g.* suppression) reveal his unconscious acceptance of current medical doctrine. His list of causes also reveals the danger of trying to isolate causes; many of the conditions which Pinel terms causes we would now consider early manifestations of illness (*e.g.* alcoholism).

[23] *Traité*, 2nd ed., p. 309. Pinel's entire medical training led him to take this attitude of non-interference with the innate curative powers of the body's economy. Drilled from his earliest years in the precepts of Hippocratic medicine, Pinel was inevitably influenced by its doctrine (*vis medicatrix naturae*) that when attacked by illness the organism marshalls all its forces to oust the disease (Max Neuburger, *The Doctrine of the Healing Power of Nature Throughout the Course of Time*, Linn J. Boyd, tr. [New York, 1933], p. 6). This faith in the body's instinct to combat disease was re-inforced by his education at Montpellier, seat of the vitalist philosophy of Bordeu and Barthez. These men propounded the doctrine that a "vital force," neither mechanical nor divine, controls the body and its health, and that this force always strives to throw off disease. Both the ancients and the vitalists had observed that in fact the body does have certain intrinsic defenses against illness, and consequently both of them opposed impulsive and unreasoned use of medications. Pinel had observed the same phenomenon, and so he reserved pharmacological methods for desperate cases in which the natural curative powers needed outside assistance.

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initiate active therapy. His methods of treatment had a simple criterion of worth: effectiveness;[24] he made no attempt to explain the entire curative process, because to do so would have required unnecessary hypothesizing.[25]

In general, Pinel's therapeutic system involved introduction of psychological techniques in place of the accepted physical methods. The first major step in Pinel's total program of moral treatment was his decision to free the patients at Bicêtre and Salpêtrière of their chains.[26] The favorable results of this experiment convinced Pinel that an important factor in achieving a cure was to accord the patient "a limited liberty in the interior of the hospital, allowing him all the movements of non-dangerous effervescence. . . ."[27]

The most vital aspect of Pinel's therapeutic program was the psycho-

[24] With his mathematical background (his M. A. thesis was entitled "De la certitude que l'étude des mathématiques imprime au jugement dans son application aux sciences") Pinel naturally saw in figures a dependable method of assessing results, and so statistical presentation of his material became one of his favorite techniques of evaluating his therapeutic observations. He advised that physicians everywhere spend less time citing their cures and more time studying the percentages of success of each treatment tested (*Traité*, 2nd. ed., p. vi). So impressed was Pinel with the worth of statistics that he published a memoir on their use (*Résultats d'observations et construction des tables pour servir à déterminer le degré de probabilité de la guérison des aliénés* [Paris: Badouin, 1808]) and tried to inspire his students to rely on the statistical method.

[25] Pinel was not opposed to "working hypotheses," without which it would be impossible to embark upon any directed scientific research. His attitude was, however, that in doing such research in medicine "one should be guided by the method which is so consistently successful in all departments of natural history, that is to say that one should observe succesively each subject at a time and do this without any intent other than to gather material for the future" (*Traité*, 2nd. ed., p. 3). He was opposed to the wild hypothesizing of so many of his predecessors who were unable to resist the temptation to explain all phenomena in the light of contrived philosophical systems. In the introduction to his work on mental illness he remarks that certain system-builders may censure him for not having engaged in "high speculations" and commented on their various theories. His reply to them was: "I could hardly cite these doctrines except as examples of aberration of the reason and of delirium, and in order to preserve the peace I have taken the wise course of passing them in silence" (*Ibid.*, p. 9).

[26] For over a century and a half, historians have referred to the "geste" of Pinel in freeing the patients of Bicêtre and Salpêtrière of their chains. Gregory Zilboorg (*A History of Medical Psychology* [New York: W. W. Norton, 1941], pp. 319-341) gives a graphic but over-glamorized description of this event, leaving the reader with the erroneous impression that, braving the Revolutionary government, Pinel freed all the inmates immediately and completely. In actual fact, Pinel observed the patients confined within the hospital and judiciously selected a few upon whom to test the effect of unshackling. He substituted strait-jackets for their chains and was careful at all times to have several strong attendants nearby to overpower the patient if he seemed dangerous. With experience he learned that most patients could be allowed complete liberty within the hospital, and so he discarded the use of strait-jackets as well as of chains in most cases.

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logical, and fundamental to this approach was kindness, combined with firmness and supported if necessary by strategy or coercion.[28] Pinel's basic therapeutic principle was his belief that:

> in the greatest number of instances . . . the experience of every day attests the value of consolatory language, kind treatment, and the revival of extinguished hope. Severity in cases of this description can answer no other purpose than those of exasperating the disease, and of frequently rendering it incurable.[29]

Believing thus, he attempted in all cases to effect a cure by gaining the patient's confidence in his good intentions and then convincing the patient that he should not lose hope, that recovery was still possible. In many cases, however, it was necessary to temper benevolence with discipline; Pinel could not be completely "permissive" with his patients, because in his opinion they had to learn self-control, and the way to learn self-control was to be reprimanded when disobedient.[30]

Some cases, however, were not readily amenable to this disciplinary benevolence and consequently required strategy or coercion. Strategy was utilized for patients suffering from delusions dangerous to themselves or to others. In such cases, the superintendent might try merely to subdue the patient by capitalizing on the delusion itself, or he might attempt to create a situation which, by suggestion, would force the patient to see the error of his delusion and reject it.[31] Coercion was used on the most recalcitrant patients and was intended to cow them into docility with a display of force without actually harming them. The fear aroused in the patient would enable the superintendent to dominate him but would be therapeutically useless unless the patient realized the reason for it and respected the authority behind it. Restraints such as strait-jackets and isolation rooms were also part of Pinel's coercive technique and administered in the same way as the show of force, that is, kept in use only until the patient became calm and obedient.

[28] It is interesting to note that Pinel himself did not do most of the work with the patients. He preferred to remain apart, observing and directing, while his lay superintendent dealt with the patients. This superintendent, a man named Pussin, was already at Bicêtre when Pinel came in 1793. A congenial relationship sprang up between the two men, and they worked together to study and treat the patients. Although he had had little formal education or training, Pussin had a natural talent for psychiatric work which made him invaluable to Pinel. Like Pinel, he disapproved of the use of chains; according to one source he had already experimented with less barbarous methods of restraint before Pinel's appointment (Pariset, *op. cit.*, p. 227).

[29] Davis Translation, p. 100.

[30] *Ibid.*, pp. 103-106.

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Pinel attempted to re-inforce these various psychological techniques by carefully supervising the patient's mode of life and his social contacts, and he strongly emphasized the value of hospitalization for both these purposes. The insane person should receive good physical care, being as well fed and kept as generally comfortable as possible, he should lead a regular life according to a strict schedule, and he should be secluded among those in a similar condition, because indiscriminate contact with others was harmful to him. Consequently, since friends and relatives had been observed to be the people most likely to augment his derangement, the patient should be hospitalized. Throughout his time in the hospital, moreover, the patient should live among people suffering from the same type of illness in the same degree, and he should be shielded at all times from the gibes of idly curious strangers.

In addition to adopting certain psychological techniques and abandoning the use of chains, Pinel also discarded the indiscriminate use of medication for insanity. His researches had proven to him that heavy medication only interfered with recovery in most cases.[32] When he did prescribe medicine, Pinel usually administered doses which were milder than those in general use; he used purgatives and emetics on occasion, because he had observed the " very active influence " [33] which the condition of the stomach had upon the deranged mind, but he used only the more gentle drugs rather than the powerful ones so admired by the ancients. Sedatives also held a subsidiary place in Pinel's program of treatment; he would use them to calm a patient to the point where he could be introduced to moral treatment. Pinel roundly condemned the common practices of beating and bleeding the insane. In his opinion, both corporal punishment [34] and bloodletting [35] were more likely to harm than help the patient.

Although Pinel abandoned many of the commonly-accepted physical methods of treating the insane, he did popularize warm baths and occupational therapy. He observed that a warm bath was invariably helpful in calming patients, and believed that in a few cases a sudden brief shower of cold water upon the head at the end of the bath might render a patient more docile. He was also among the first to include work in his therapeutic program. He had the greatest faith in its value to the patient,[36] and therefore bent all his efforts to institute such a system at Bicêtre and Salpêtrière, giving patients work provided by various Paris merchants and hiring some convalescents as attendants.

Pinel's conception of treatment for mental illness included all the aspects

[32] Davis Translation, p. 68.
[33] *Traité*, 2nd. ed., p. 358.
[34] *Ibid.*, p. 254.
[35] *Ibid.*, pp. 320, 476; Davis Translation, p. 251.
[36] *Traité*, 2nd. ed., p. 239.

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Pinel attempted to re-inforce these various psychological techniques by carefully supervising the patient's mode of life and his social contacts, and he strongly emphasized the value of hospitalization for both these purposes. The insane person should receive good physical care, being as well fed and kept as generally comfortable as possible, he should lead a regular life according to a strict schedule, and he should be secluded among those in a similar condition, because indiscriminate contact with others was harmful to him. Consequently, since friends and relatives had been observed to be the people most likely to augment his derangement, the patient should be hospitalized. Throughout his time in the hospital, moreover, the patient should live among people suffering from the same type of illness in the same degree, and he should be shielded at all times from the gibes of idly curious strangers.

In addition to adopting certain psychological techniques and abandoning the use of chains, Pinel also discarded the indiscriminate use of medication for insanity. His researches had proven to him that heavy medication only interfered with recovery in most cases.[32] When he did prescribe medicine, Pinel usually administered doses which were milder than those in general use; he used purgatives and emetics on occasion, because he had observed the " very active influence " [33] which the condition of the stomach had upon the deranged mind, but he used only the more gentle drugs rather than the powerful ones so admired by the ancients. Sedatives also held a subsidiary place in Pinel's program of treatment; he would use them to calm a patient to the point where he could be introduced to moral treatment. Pinel roundly condemned the common practices of beating and bleeding the insane. In his opinion, both corporal punishment [34] and bloodletting [35] were more likely to harm than help the patient.

Although Pinel abandoned many of the commonly-accepted physical methods of treating the insane, he did popularize warm baths and occupational therapy. He observed that a warm bath was invariably helpful in calming patients, and believed that in a few cases a sudden brief shower of cold water upon the head at the end of the bath might render a patient more docile. He was also among the first to include work in his therapeutic program. He had the greatest faith in its value to the patient,[36] and therefore bent all his efforts to institute such a system at Bicêtre and Salpêtrière, giving patients work provided by various Paris merchants and hiring some convalescents as attendants.

Pinel's conception of treatment for mental illness included all the aspects

[32] Davis Translation, p. 68.
[33] *Traité*, 2nd. ed., p. 358.
[34] *Ibid.*, p. 254.
[35] *Ibid.*, pp. 320, 476; Davis Translation, p. 251.
[36] *Traité*, 2nd. ed., p. 239.

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mentioned here; one must not think that he applied only one method of therapy to each patient. Although he was strongly opposed to combining diverse techniques simultaneously and at random, he realized that as a patient's condition changed the treatment applied to him should change accordingly. He placed great reliance upon psychological methods, but was willing to use other techniques if they seemed necessary.

Not only did Pinel put his therapeutic principles into effect at Bicêtre and Salpêtrière; he also set them forth in his book entitled *Traité médico-philosophique sur l'aliénation mentale.*[37] With great clarity and relatively little theoretical bias, Pinel described in it his work with the mentally ill and, both by direct exposition and by example, his observational technique. He described various cases and demonstrated that since it could be cured within a hospital setting with proper treatment, mental illness was most certainly a problem for the physician rather than for the jailer or the priest. Since his time psychiatry has been a medical speciality.

In addition to describing his psychiatric work in this volume, Pinel also introduced many young men to psychiatry by working with them at Salpêtrière. His most famous pupil was Jean-Etienne-Dominique Esquirol; Esquirol in turn passed Pinel's precepts on to another entire generation of French psychiatrists—such men as Georget, Ferrus, Falret, Voisin, and Trélat.

It is not so much for his famous "geste" of unchaining certain unfortunates, therefore, that Pinel deserves the fame he has today. Other men did similar work independently and simultaneously. Pinel merits recognition for having abandoned cumbersome hypotheses and out-moded prejudices in his medical research and for having popularized a new therapeutic approach to insanity. Very much a product of the eighteenth-century enlightenment, Pinel wished to apply a new technique of science to a new field of medicine. His sincere attempt to be objective in his research, the humane treatment which he applied to his patients, and the clarity with which he set forth what he had observed and concluded would make him much admired among scientists and physicians today.

ACKNOWLEDGMENT: The authors would like to thank Norman Dain and Valerie Schwarzchild for their assistance in the preparation of this paper.

[37] The first edition of this work appeared in 1801. In the same year it was translated into German by Mich. Wagner (*Philosophischmedicinische Abhandlung über Geistesverirrungen oder Manie* [Vienna: C. Schaumburg]); in 1804 Luis Guarnerio y Allavena produced a Spanish version (*Tratado médico-filosófico de la enajenación del alma o mania* [Madrid: Imprenta Real]), and finally in 1806 D. D. Davis brought out his English version. In 1809 the second edition appeared. The authors have also seen two references to a third edition of around 1818, but have not been able to verify them.

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